

Understanding COPD Exacerbations Through a Patient Lens Qualitative Research

On behalf of GSK, Just Jacqs conducted research with 18 participants using mini groups and one-on-one depth interviewing

BACKGROUND TO THIS RESEARCH

- A focus on “exacerbations” as a clinical term doesn’t feel very patient friendly language. GSK hypothesised this term, might result in patients not recognising their symptoms as something they can address.
- Disease management confusion might hinder them talking to their Doctor about the need to adjust or step up their COPD course of treatment.
- Understand how to facilitate easier conversations for patients who did wish to talk to their doctor or whanau about their symptoms.
- Understand how patients with moderate to severe COPD talk about their symptoms, disease management and treatment conversations.

RESEARCH OBJECTIVES

Primary

- Understand the patient and their day to day lives
- Determine patient centric language, and messaging that is relatable, motivating and salient for those with moderate to severe COPD
- Understand how COPD patients refer to their “exacerbations
- Enhance meaning and engagement moving forward

Secondary

- Uncover current pathways for patients with moderate to severe COPD to get the right treatment
- Understand enablers and barriers to treatment pathways
- Explore educational/informational wrap around needs that can support sector leadership and patient advocacy

QUALITATIVE METHODOLOGY

- Independent research
- Focus groups and one on one depth interviews
- Spread of locations, gender, ethnicities and age
- Blind recruitment
- Voluntary participation
- All participants had been diagnosed with COPD.
- All participants on one or more inhalers currently
- Participants agreed to one the following subjective statements with a spread across sample.
- I currently feel I have my COPD under control and I’m managing it well
- It’s hard to know if I have my COPD under control or if I’m managing it well
- I don’t feel I have my COPD under control or managing it as well as I’d like



Key Findings

PATIENT COPD LITERACY IS LOW

DEGREES OF MISINFORMATION

- Confusion as to whether COPD is 'a label', 'a condition', 'a disease' or 'a diagnosis'
- The uncertainty about what COPD stands for is a barrier to having informed conversations
- The collection of symptoms adds confusion. The presentation of disease symptoms is individual.
- Prescriptions or scripts do not always come with explanations
- Information, education and awareness surrounding COPD was inconsistent and idiosyncratic across the research participants.
- COPD feels scary and untreatable which makes it harder to realise effective management is available.

"I worked out I must have COPD because of the types of inhalers I'm on"

"I don't know what the letters mean, and no one has told me. It was hard to tell my daughters that I had this thing because I didn't know what it was"

"There is no good conversation you can have about COPD. Nothing good will come out of it ..."

A DIAGNOSIS MATTERS

CONSIDERABLE SOCIAL, EMOTIONAL AND FINANCIAL COST OF NON-MANAGED COPD

- Avoidance of others – including conversations.
- Higher rates of other health related issues.
- Poor outcomes, such as increased healthcare utilisation and poorer quality of life.
- Isolation.
- Social stigma.
- Reduced activities and community connection.
- Decreased productivity – reliance on others to do more and loss of income
- Escalating health insurance
- Strain on the health system

"Getting a diagnosis has given me more support, because I've gone from just being a chronic asthmatic and getting prescribed the same medication over and over, without getting to the bottom of why I couldn't do things more easily. Now I know"

"I don't really leave the house much these days as just getting ready to go out exhausts me".

"I do feel isolated and like I'm missing out on life"



WELL MANAGED COPD IS SUBJECTIVE

'MANAGED COPD' CAN OFTEN BE NON-MANAGEMENT IN DISGUISE.

- Dismissing COPD as their 'everyday normal'.
- An over reliance on their inhaler
- Relying on breathing or calming techniques to help with exacerbations.
- Managing in silence – being stoic.
- Avoiding external irritants/variables or changes in routines.
- Regulating temperature.
- Second guessing and denial – it's age, it's a cold, it's just me
- Patients are unsure how managed vs non managed COPD would present, put up with a lot.

"I've just accepted it's who I am now. What can you do"

"I can't do as much anymore. Just no energy"

"I try and calm myself down by removing any over stimulation"

"I trust if my doctor needs to talk to me about it then he will"

"If I'm not in hospital then I guess it's managed,"

"I wouldn't go to my Doctor about Hay Fever so why go about my COPD?"

"Who can control the weather?"

BARRIERS TO EFFECTIVE TREATMENT OUTCOMES

THERE IS AN UNHELPFUL SILENCE SURROUNDING THE DISEASE

- Few open or informed conversations away from clinical environments.
- The range of emotions patient are grappling with – fear, shame, guilt, sadness and embarrassment – they often hide or mask their COPD and their symptoms from their families.
- Diagnostic overshadowing means COPD is lower down in priority or overshadowed by other health issues. Often a host of things a patient is trying to talk to their Dr about on a visit. COPD is not always top of mind.
- Having one consistent GP that is aware of your medical history.

"I never talk to my family about it – it upsets them and they get sad for me. I don't want them to worry about me. I just avoid talking about it."

"The Doctor has never proactively talked to me about how I manage my COPD. Or what the next option is for treatment or anything. I'd like to know that"

"I hate it when someone asks are you alright? when I'm trying to catch my breathe"

"My Doctor never talks to me about my COPD, so I assume it's because it's relatively well managed"

The masking and 'behind closed doors' nature of COPD is a big opportunity to help normalise and destigmatize the disease. This will open up pathways to better treatment outcomes.



MOVING FROM UNMANAGED TO STABLE COPD

FROM

- Everyday tasks feels laboured
- Avoiding others and feeling isolated
- Masking your symptoms from others
- Feeling embarrassed
- Not able to work
- Reduced physical activity
- Feeling you need to use your medicine more than prescribed

TO

- Able to complete day to day task
- Socialising & taking part in leisure activities
- Increased self confidence
- Continue to work
- Everyday feels better and secure
- Able to stay physically active
- Taking medicine at the prescribed level

KEY LEARNINGS

1. **COPD health literacy is low.** There is limited recall of helpful, consistent information and collective understanding. The varied levels of patient misinformation, confusion and ambiguity suggests clarity of terminology and language will provide value.
2. **'Managing' COPD can often be non-management in disguise.** Framed as putting up with their 'everyday normal', prescription status quo or inconsistent application of best practice frameworks.
3. **Unmanaged COPD has human, social and financial impact.** The reduced quality of life and increased strain on wider support systems – be they family, health providers or the health system itself is evident.
4. **COPD needs to be more than an acronym.** Hearing and seeing what COPD stands for – Chronic Obstructive Pulmonary Disease – is helpful to increase disease awareness and understanding. Increasing understanding of COPD as a disease will help patients feel they don't have to battle on in silence and to feel more supported.
5. **COPD can feel self-inflicted.** People are living with more fear, uncertainty and self judgement than they need to. Guilt, is limiting their treatment pathway conversations and to better disease management.
6. **Communication that allows patients to feel understood and valued can reduce stigma.** Continuing to nudge patients to expect more help and to know what and who is there to help, will help overturn the resignation and shame surrounding COPD management.
7. **Patients need to better communication about what flare ups mean for them.** Equally, patients need to be able to more easily access best practice and emerging knowledge around what non managed COPD looks like and across the full range of exacerbations. For Doctors and patients alike, exacerbations are viewed as serious and acute rather than progressive and constant. This inhibits the documenting of the full spectrum of exacerbations.
8. **Doctor led conversations are more likely to lead to treatment step ups than patient led ones.** Enabling two-way dialogue and providing information for Doctors to provide to their patients at the time of prescribing will be valuable.
9. **Continued disease awareness and education is fundamental. This will enable patients to have ongoing conversation with their GP, regardless of who is the initiator. In this instance, HCP enablement will come before patient choice and empowerment.**

